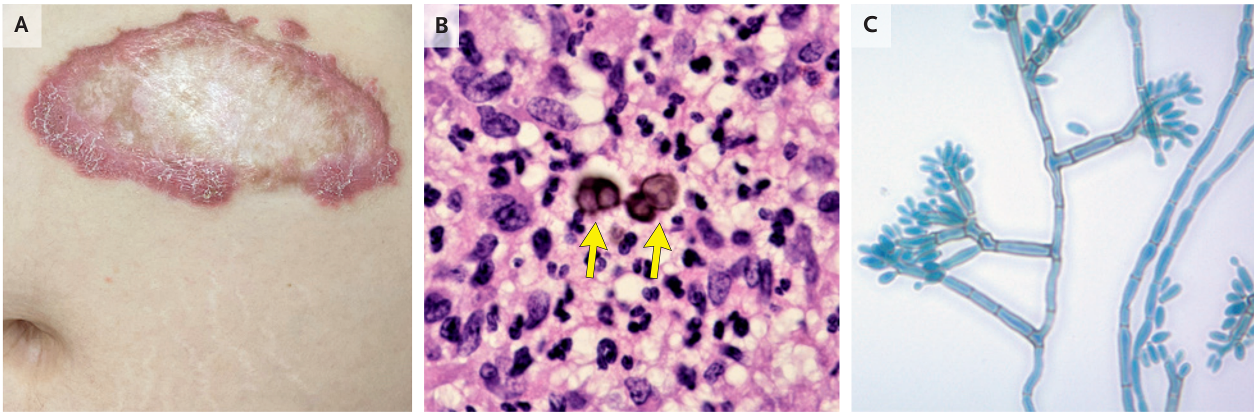
IMAGES IN CLINICAL MEDICINE

Chromoblastomycosis



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A 38-year-old pregnant woman at 30 weeks' gestation was referred to the dermatology clinic for evaluation of a long-standing, itchy lesion on her abdomen. The lesion had first appeared 9 years earlier after the patient had emigrated from Honduras, where she had worked in agriculture. The lesion had subsequently slowly expanded. On physical examination, a 15-cm erythematous, scaly plaque with a raised border and atrophic center was visible on the left upper abdomen (Panel A). Histopathological analysis of a skin-biopsy sample showed pseudoepitheliomatous hyperplasia with dermal granulomatous infiltrates. Thick-walled, brown, septate spores called muriform cells (Panel B, arrows) were also seen. The presence of such cells (also known as Medlar bodies, sclerotic bodies, or copper pennies) is pathognomonic for chromoblastomycosis. Fungal culture grew a pigmented mold, which showed septate hyphae with loosely branched clavate conidia and prominent denticles on microscopy (Panel C). The causative organism was identified as a fonsecaea species. A diagnosis of chromoblastomycosis with cicatricial morphologic features was confirmed. Chromoblastomycosis is a chronic, progressive implantation mycosis caused by dematiaceous fungi in soil and plant matter. It is typically diagnosed in agricultural workers in tropical or subtropical regions. Postpartum treatment with itraconazole was planned, but the patient was lost to follow-up.

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